INDIVIDUAL CASE SAFETY REPORT (ICSR)

Task 5 – Final Project

1. PATIENT AND CASE INFORMATION

Patient initials / name	R.P.
Age / sex / weight	32 years / Male / 65 kg
Diagnosis	Acute bacterial sinusitis
Known allergy history	No history of allergy reported
Case source	Task 2 case scenario
Report type	Initial educational practice ICSR

2. ADR IDENTIFICATION AND MEDDRA CODING

Reported reactions	Generalized itching and skin rash over the chest
Simple MedDRA coding	The patient reported itching and a skin rash. For coding, “itching” can be recorded as “Pruritus” and “skin rash” as “Rash”, using the closest matching MedDRA terms.
Onset / recovery	03 March 2026 / 06 March 2026
Outcome	Recovered completely
Note	These are simple medical terms for the reported symptoms. If formal MedDRA coding is required, the exact terms can be checked in the MedDRA browser.

3. SUSPECTED MEDICINAL PRODUCT

Suspected drug	Amoxicillin
Dose / route / frequency	500 mg / Oral / Three times daily (TID)
Indication	Acute bacterial sinusitis
Start / stop date	01 March 2026 / 03 March 2026
Planned duration	5 days
Batch / manufacturer / expiry	Not provided
Action taken / Dechallenge	Amoxicillin was stopped on 03 March 2026. The patient recovered by 06 March 2026 after stopping the medicine and receiving antihistamines
Rechallenge	Not reported

4. CONCOMITANT MEDICATION AND MANAGEMENT

Other medicines	No other medicine was reported before the reaction.
Reaction treatment	Antihistamines were given; name, dose and dates were not provided.
Investigations	Not provided

5. REPORTER INFORMATION

Reporter	Sakshi Saket (student)
Location	Patna, Bihar, India
Contact details	Not included in this educational copy
Report date	30 September 2026
Source	Task 2 case scenario supplied for the assignment

6. CAUSALITY ASSESSMENT

Time relationship	Yes. The reaction started two days after amoxicillin was started.
After stopping the drug	The patient recovered after amoxicillin was stopped. Antihistamines were also given, so their effect cannot be separated.
Other possible causes	The case does not provide enough information to assess other possible causes.
Rechallenge	Not reported.
WHO–UMC assessment	Possible. The timing supports a possible link with amoxicillin, but other causes cannot be ruled out and rechallenge information is unavailable.

7. SERIOUSNESS AND EXPECTEDNESS CLASSIFICATION

Seriousness	Non-serious based on the details provided. The case does not report hospitalization, a life-threatening event, death, disability or another serious outcome.
Expectedness	Rash and itching are known possible reactions to amoxicillin. Check the applicable product information to confirm expectedness.
Overall classification	Non-serious suspected ADR; potentially expected (to be checked against product information); WHO–UMC causality: Possible.

8. CASE NARRATIVE

A 32-year-old male patient weighing 65 kg was prescribed amoxicillin 500 mg orally three times daily for five days for acute bacterial sinusitis. He started treatment on 01 March 2026. On 03 March 2026, he developed generalized itching and a skin rash over the chest. Amoxicillin was stopped on the same day and antihistamines were given. The patient recovered completely by 06 March 2026. No previous history of allergy was reported. Information about investigations, the specific antihistamines, other possible causes and rechallenge was not available. Amoxicillin is considered the suspected medicine. Based on the available information, causality is assessed as Possible.

9. FINAL ASSESSMENT

This ICSR covers the four key deliverables of Task 5: ADR identification, reporting, causality assessment, and seriousness/expectedness classification. It is based on one patient case. Details not included in the case are marked “Not provided” rather than assumed.